

Surgery for oesophageal cancer

If the oesophageal cancer is at an early stage, you may have surgery with the aim of curing the cancer.

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About surgery for oesophageal cancer

Surgery for [oesophageal cancer](#) is done by specialist surgeons. It is not available in all hospitals, so you may need to travel to a hospital further away to have the operation.

Oesophagectomy

The main operation used to treat oesophageal cancer is called an [oesophagectomy](#).

In this operation, the part of the [oesophagus](#) containing the cancer is removed. The part of the oesophagus that is removed depends on the size and position of the cancer inside the oesophagus.

You may need to stay in hospital for about 2 weeks after surgery to the oesophagus. Some people may stay longer.

Surgery for early stage oesophageal cancer

Some very early stage oesophageal cancers are only in the inner layer of the [oesophagus](#). Only a small number of people are diagnosed with very early stage oesophageal cancer.

Early stage oesophageal cancers can be treated using the following treatments:

- Endoscopic mucosal resection (EMR) – this removes small cancers on the inner layer of the oesophagus called the [mucosa](#).
- Endoscopic submucosal dissection (ESD) – this can remove slightly larger cancers that may be growing into the layer of tissue below the mucosa, called the [submucosa](#).

The doctor passes an [endoscope](#) through the mouth and into the oesophagus, so they can see the cancer. They then inject a small amount of fluid into the layer of cells below the cancer. The fluid lifts the cancer up from the muscle layer in the oesophagus. This makes it easier to remove the cancer. The doctor does this using suction and a small knife or wire loop called a snare.

The most common side effects are bleeding and narrowing of the oesophagus. There is a very small risk of a tear (perforation) in the wall of the oesophagus.

Further treatment

Your cancer doctor may recommend further treatment after an EMR or ESD. This is to reduce the risk of the cancer coming back. For example, they may suggest you have a treatment called radiofrequency ablation (RFA).

You do not usually have a general anaesthetic for EMR or ESD but you may be given medicine to numb the throat. Some people may be given medicine to make them drowsy (sedative). Your doctor or nurse will talk to you about the preparation and recovery for this type of surgery.

Recovering after your operation

Before you leave hospital, you will be given an appointment for your check-up at an outpatient clinic. You may also be given contact details for your specialist nurse. The appointment is a good time to talk about any problems you have after your operation. If you have any problems before this appointment, you can contact your cancer doctor, specialist nurse or ward nurse for advice.

We have more information about recovering after an [oesophagectomy](#).

About our information

This information has been written, revised and edited by Macmillan Cancer Support's Cancer Information Development team. It has been reviewed by expert medical and health professionals and people living with cancer.

References



Below is a sample of the sources used in our oesophageal cancer information. If you would like more information about the sources we use, please contact us at informationproductionteam@macmillan.org.uk

European Society for Medical Oncology. Oesophageal Cancer: ESMO clinical practice guidelines for diagnosis, treatment and follow up. 2022. Available from www.esmo.org/guidelines/guidelines-by-topic/gastrointestinal-cancers/oesophageal-cancer [accessed July 2023].

National Institute for Care and Health Excellence. Oesophago-gastric cancer: assessment and management in adults NICE guideline [NG83]. 2018. Available from www.nice.org.uk/guidance/ng83 [accessed July 2023].

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Our cancer information meets the [PIF TICK](#) quality mark.

This means it is easy to use, up-to-date and based on the latest evidence. Learn more about [how we produce our information](#).

The language we use

We want everyone affected by cancer to feel our information is written for them.

We want our information to be as clear as possible. To do this, we try to:

- use plain English
- explain medical words
- use short sentences
- use illustrations to explain text
- structure the information clearly
- make sure important points are clear.

We use gender-inclusive language and talk to our readers as 'you' so that everyone feels included. Where clinically necessary we use the terms 'men' and 'women' or 'male' and 'female'. For example, we do so when talking about parts of the body or mentioning statistics or research about who is affected.

You can read more about how we produce our information [here](#).

How we can help

Macmillan Support Line

The Macmillan Support Line is a free and confidential service for people living with and affected by cancer. If you need to talk, we'll listen.



Call us free on 0808 808 0000

8am to 8pm, 7 days a week. You can talk to our cancer specialists about anything.



Email the Macmillan Support Line

Email the Macmillan Support Team if you have any questions.

Chat online

Available 8am to 7.45pm, 7 days a week. For confidential, real-time support if you don't want to call.



Online Community

Chat online anonymously to others who understand what you are going through. Our community is available 24/7 and has dedicated forums where you can get advice and ask our experts.



Find cancer support services in your area

What's going on near you? Find out about support groups, where to get information and how to get involved with Macmillan where you live.

Get help